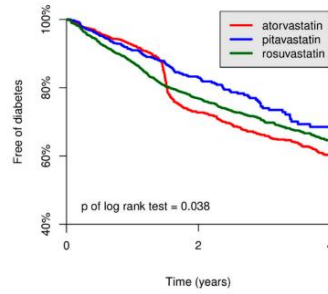


Choosing the Right Statin: A Comparative Evidence Review

Pitavastatin vs. Atorvastatin vs. Rosuvastatin: The Statin Debate

Parameter	Pitavastatin	Atorvastatin	Rosuvastatin
New-Onset Diabetes Mellitus (NODM) Risk¹	28% lower risk vs. Atorvastatin & Rosuvastatin.	45% higher risk compared to Pitavastatin.	35% higher risk compared to Pitavastatin.
Diabetes-Free Survival (4-Year Follow-up)²	<i>Higher probability of remaining NODM-free vs. Atorvastatin & Rosuvastatin ($p = 0.038$).</i>  Kaplan-Meier plot of three statin groups for new-onset diabetes mellitus (NODM)-free probabilities curve		
Lipid & CV Protection	↓ hs-CRP 28.6% (62.4% in high-risk)³ - Better LDL-C reduction, plaque regression, HDL-C boost vs. Atorvastatin⁴ ~64% lower CV risk vs. Atorvastatin⁵	- Less effective in inflammation reduction & plaque regression⁴ - Higher CV risk (8.1%)⁵	~10% HDL increase⁶ - Effective, but with higher diabetes risk⁶
Renal Outcomes	- Effective in eGFR <60 ml/min/1.73 m², maintaining stability. - Lower renal risk.^{1,7}	- Declined by 15.6%–18.2%. 4.5% renal events.⁸	- Greater eGFR reduction. 7.8–9.8% renal events.⁸
Myalgia & Myopathy Risk	Lower risk due to minimal CoQ10 reduction.^{9,10}	Higher risk due to 26.1% CoQ10 depletion.⁹	Associated with CoQ10 depletion.¹



Pitavastatin ensures lower NODM risk, better CV protection, and renal stability than Atorvastatin and Rosuvastatin. It also offers effective lipid control with a lower myopathy risk, making it a well-balanced choice.

CV, Cardiovascular; CoQ10, Coenzyme Q10; eGFR, Estimated Glomerular Filtration Rate; HDL-C, High-Density Lipoprotein Cholesterol; hs-CRP, High-Sensitivity C-Reactive Protein; LDL-C, Low-Density Lipoprotein Cholesterol; NODM, New-Onset Diabetes Mellitus.

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